PC-08 Pediatric Post-Resuscitation

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Sustained ROSC in patients age 3 days through 15 years. Prevent recurrent arrest and secondary injury through controlled oxygenation, ventilation, perfusion, glucose, temperature, and rhythm management.

Field flow

1. Stabilize	Confirm pulse/perfusion and stabilize airway, breathing, circulation, glucose, temperature, and rhythm.
2. Oxygen / ventilation	Once reliable SpO2 is available, titrate toward 94-99% unless an individualized baseline applies; avoid hypocapnia and hypercapnia.
3. Perfusion	Maintain systolic and mean arterial pressure above the 10th percentile for age/sex. Avoid routine fluid boluses.
4. Temperature / glucose	Check temperature and glucose. Avoid core temperature above 37.5 C in the comatose child; correct hypoglycemia within standing orders.
5. Destination	Treat persistent dysrhythmia with the rhythm-specific pathway; notify and transport to the most appropriate pediatric-capable destination.

Provider-level actions

Level	Approved actions
EMT	Maintain airway/appropriate ventilation, titrate oxygen to reliable SpO2 94-99% unless individualized target, check glucose when authorized, prevent fever, and reassess.
AEMT	All EMT care plus IV/IO, glucose correction, cautious cause-directed fluid only for suspected hypovolemia, and authorized post-airway analgesia/sedation.
Paramedic	All prior care plus cardiac/12-lead monitoring without delay, waveform EtCO2 after advanced airway, rhythm-specific treatment, and Medical-Control-directed vasopressor support.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Do not force a usual saturation target in cyanotic congenital heart disease without individualized guidance.

• Avoid core temperature above 37.5 C in the comatose child; do not start field targeted-temperature management unless directed by Medical Control.

• Do not administer routine fluid boluses.

• If pulseless again, return immediately to PC-04; use PC-01 or PC-07 according to rhythm.

Medication reference

Medication	Approved dose / safeguard
Fluids / vasopressors	No routine bolus. Use cautious reassessed fluid for suspected hypovolemia or Medical-Control-directed vasopressor support.
Analgesia / sedation	Treat pain first, then provide age-appropriate sedation. Paralysis alone is not sedation.

Linked pathways

AR-08 Post-Intubation / BIAD Management; PC-01 Asystole / PEA; PC-04 Pulseless Arrest; PC-07 VF / Pulseless VT; Medication Reference.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.